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Ailment



Waking during surgery and feeling pain without being able to cry out. Not just a horror-movie staple. They do happen in reality.

NOT ENOUGH ANAESTHESIA

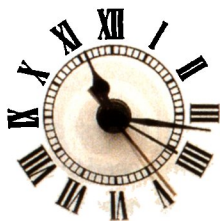
Many surgeons mention that they have faced this in the OT. Several studies have found "anaesthesia awareness" to be a 1-in-1,000 occurrence, with an estimated 20,000 to 40,000 patients waking up from general anaesthesia each year and one-fourth reporting pain. Anaesthesia typically consists of three medications: a paralytic to ensure the patient doesn't move, a hypnotic to induce unconsciousness and a narcotic to numb pain. "It's the combination of these drugs in their various doses that determines the depth of unconsciousness," says Dr G.P. Dureja, anesthesiologist and head of the Delhi Pain Management Centre. Apart from faulty drug dosage, breakdown or misuse of equipment, patient health status and lack of expensive monitoring technology can also cause anaesthesia to fail, he explains.

Prescription

Make it mandatory for hospitals to install hi-tech monitoring devices, like the bispectral index, that enable doctors to know the depth of consciousness in a patient during an operation.

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Most hospitals are not mobilised to provide timely care to patients. One can watch a game of Twenty20 cricket in the average pain-to-drug time-frame in India.

DANGEROUS DELAY

According to a recent study of cardiac patients by the medical journal, *The Lancet*, what disadvantages Indians is the time taken to access medical help. Much of the delay happens because of the patient's inaction (60 to 70 per cent). But most hospitals, it seems, are not mobilised to provide timely care. "At the hospital, a patient, on an average, wastes 50 minutes to get treated, compared to a waiting time of 20 to 30 minutes in the developed world," says Dr K.S. Reddy, former head of cardiology in AIIMS and one of the authors of the study.

Prescription

Hospitals must have well-equipped and functional emergency wards and must provide ambulance services with lifesaving equipment and a doctor on board.

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Overuse and misuse of unnecessary drugs. International norms are flouted as banned, untested and untried combinations do the rounds in India.

UNNECESSARY DRUGS

The World Health Organisation recommends 250 essential drugs, India allows 70,000. No wonder, the Delhi Society for Promotion of Rational Use of Drugs has found a massive misuse of antibiotics in hospitals. "Drugs that are banned in the developed world are often widely used here," says Dr Ambrish Mithal, endocrinologist at Delhi's Apollo Hospital. "For diabetes we have pills with three different medicines rolled into one," he says. "These combinations are not tried and tested. All the ingredients need to be sold separately, as it happens in other countries."

Prescription

The Central Drugs Standard Control Organisation (CDSCO) must clamp down on misuse of untested combinations and penalise offending doctors and pharmacies.